MODULE 4

Strategy is not a meeting. It's a daily discipline of asking: where are we going, and is today's work pointing there?

Most healthcare managers are excellent operators. Far fewer are genuine strategic thinkers.

1 ENVIRONMENTAL SCANNING

The biggest operational problems in your practice right now were predictable 12 months ago — if anyone had been looking.

- PESTLE: Political, Economic, Social, Technological, Legal, Environmental.
- CMS reimbursement changes, labor market shifts, patient expectation evolution — these aren't surprises. They're signals.
- The practice that scans systematically plans proactively. The one that doesn't reacts perpetually.

KEY INSIGHT
What is the most significant external force currently affecting your practice that you haven't formally addressed?

Seven priorities is not a strategy. It is a list that will exhaust your team and produce nothing.

- The Vision-to-Action Cascade connects a three-year vision to what you do next Tuesday.
- Three priorities maximum. The discipline of choosing is where the strategic thinking actually lives.
- The Urgent-Important Matrix: the most important not-urgent work gets displaced by the merely urgent every week.

KEY INSIGHT
What is the most important thing your team is not doing right now because urgent things keep winning?

The problem that keeps coming back was never properly defined the first time.

- Most solutions address the symptom. Root cause solutions stick.
- The 5 Whys: ask why five times in sequence. Why 5 is almost always the root cause.
- Before proposing any solution, you need to be able to answer: what problem are we actually solving?

KEY INSIGHT
Name a recurring problem in your practice. How many times have you solved it? Has it come back? That's the diagnostic.

4

PLANNING & EXECUTION

Most initiatives don't fail from poor execution. They fail from planning that never asked the hard questions.

- Every initiative needs one owner, success metrics, resources, timeline, and a Pre-Mortem.
- The Pre-Mortem: imagine failure before it happens. It surfaces risks forward-planning consistently misses.
- 'Three people are leading this' is the same as 'no one is accountable for this.'

KEY INSIGHT

What is your most important current initiative? Can you name the single person accountable for its outcome?

A strategy that only lives in the manager's head produces compliance at best — and quiet resistance at worst.

- The Commitment Spectrum runs from resistance through compliance to enrollment to genuine ownership.
- Moving team members from compliance to enrollment requires two things: involvement and connection to purpose.
- People don't own what they didn't help build.

KEY INSIGHT
Which team members are complying with your current strategy, and which ones genuinely own it? What produced the difference?

FRAMEWORK

The 5 Whys — Root Cause in Five Questions

The same five questions applied to any persistent problem will almost always find the real cause.

1. Why 1: Why is the presenting problem occurring? (This is usually still a symptom.)
2. Why 2: Why does that happen? (Getting closer.)
3. Why 3: Why does that happen? (This is where most people stop — too early.)
4. Why 4: Why does that happen? (You're close.)
5. Why 5: Why does that happen? (This is almost always the root cause — and it's usually a system, not a person.)

REFLECT ON THIS

You can't fix what you don't accurately diagnose.

Think of a problem in your practice that has been solved more than once. What would happen if you asked 'why' five times instead of going straight to solutions?

TRY THIS

Three strategic actions for this week:

1. Apply the 5 Whys to one recurring problem in your practice. Write what you find at Why 5.
2. Identify the one initiative on your plate with no clear single owner. Assign one.
3. Name the three things your practice is prioritizing this year. Can every team member name them too?

The Focused Leader

Module 4 · Strategic Thinking & Planning

More on Module 4:
Strategic Thinking & Planning
Follow for the full series.

thefocusedleader.coach